

PATIENT MEDICAL RECORD

MEMORIAL GENERAL HOSPITAL

Department of Internal Medicine

PATIENT INFORMATION

Patient ID: MED-2025-4792

Name: Michael Thompson

Date of Birth: March 15, 1978 (47 years old)

Gender: Male

Blood Type: O+

MRN: 847293

Insurance: Blue Cross PPO

VISIT DATE: September 4, 2025

Provider: Dr. Sarah Mitchell, MD

Visit Type: Comprehensive Follow-up

CHIEF COMPLAINT

"Persistent fatigue and occasional chest discomfort during exercise"

ALLERGIES

Penicillin - rash

Sulfa drugs - GI upset

HISTORY OF PRESENT ILLNESS

Mr. Thompson is a 47-year-old male with a history of Type 2 Diabetes Mellitus and hypertension who presents for a 3-month follow-up. He reports increased fatigue over the past 6 weeks, particularly in the afternoons. He also notes mild chest tightness during his morning walks, which resolves with rest after 2-3 minutes. No radiation of pain, no nausea or diaphoresis. The patient denies orthopnea, PND, or lower extremity edema.

PAST MEDICAL HISTORY

1. Type 2 Diabetes Mellitus - diagnosed 2019
2. Hypertension - diagnosed 2020
3. Hyperlipidemia - diagnosed 2021
4. GERD - diagnosed 2018
5. Seasonal allergies

MEDICATIONS

1. Metformin 1000mg PO BID
2. Lisinopril 20mg PO daily
3. Atorvastatin 40mg PO daily
4. Omeprazole 20mg PO daily
5. Cetirizine 10mg PO PRN

SOCIAL HISTORY

- Occupation: Software engineer (sedentary work)
- Tobacco: Former smoker, quit 2015 (10 pack-year history)
- Alcohol: Social, 2-3 drinks per week
- Exercise: Walks 30 minutes daily
- Diet: Trying to follow diabetic diet, admits to occasional non-compliance

FAMILY HISTORY

Father: CAD, MI at age 62
Mother: Type 2 DM, HTN
Brother: Healthy
Children: 2, healthy

REVIEW OF SYSTEMS

Constitutional: Positive for fatigue, no fever or weight loss
Cardiovascular: Positive for exertional chest discomfort
Respiratory: Negative for SOB, cough
GI: Occasional reflux, controlled with PPI
Endocrine: Blood sugars "running higher than usual"

PHYSICAL EXAMINATION

Vital Signs:

- BP: 138/88 mmHg
- HR: 78 bpm, regular
- RR: 16/min
- Temp: 98.6°F
- O2 Sat: 98% on room air
- BMI: 29.8 kg/m²

General: Alert, oriented, mildly fatigued appearing
HEENT: PERRLA, EOMI, no thyromegaly

Cardiovascular: RRR, no murmurs/rubs/gallops, no JVD
Pulmonary: CTAB, no wheezes/rales/rhonchi
Abdomen: Soft, non-tender, no organomegaly
Extremities: No edema, pulses 2+ bilaterally
Neurological: A&O×3, CN II-XII intact

LABORATORY RESULTS (Today)

HbA1c: 8.2% (elevated, previous 7.4%)
Fasting Glucose: 156 mg/dL (elevated)
Total Cholesterol: 218 mg/dL
LDL: 142 mg/dL (elevated)
HDL: 38 mg/dL (low)
Triglycerides: 190 mg/dL
Creatinine: 1.1 mg/dL
eGFR: 72 mL/min/1.73m²
TSH: 2.8 mIU/L (normal)
CBC: Within normal limits
Troponin: Negative

ECG: Normal sinus rhythm, no ST changes, no previous comparison

ASSESSMENT & PLAN

-
1. EXERTIONAL CHEST DISCOMFORT - Concerning for possible stable angina
 - Order cardiac stress test
 - Start Aspirin 81mg daily
 - Refer to cardiology for further evaluation
 - Patient advised to stop activity if chest discomfort occurs
 2. TYPE 2 DIABETES MELLITUS - Suboptimal control
 - Increase Metformin to 1000mg TID if tolerated
 - Consider adding SGLT2 inhibitor if no improvement
 - Refer to diabetes educator for dietary counseling
 - Recheck HbA1c in 3 months
 3. HYPERTENSION - Not at goal
 - Increase Lisinopril to 30mg daily
 - Home BP monitoring recommended
 - DASH diet education provided
 4. HYPERLIPIDEMIA - Above target
 - Continue Atorvastatin 40mg
 - Emphasized dietary modifications

- Recheck lipid panel in 6 weeks

5. FATIGUE

- Likely multifactorial (poor glycemic control, possible cardiac etiology)
- Check Vitamin D, B12 levels
- Sleep study if symptoms persist after other issues addressed

FOLLOW-UP

- Cardiology appointment scheduled for next week
- Return to clinic in 4 weeks after cardiac workup
- Call if chest pain worsens or becomes more frequent
- Patient instructed to go to ED if severe chest pain

PATIENT EDUCATION

Discussed importance of:

- Medication compliance
- Daily blood glucose monitoring
- Cardiac warning signs
- Importance of stress test

Patient verbalized understanding of plan and agreed to follow-up as scheduled.

Electronically signed by: Sarah Mitchell, MD

Date/Time: September 4, 2025, 14:30